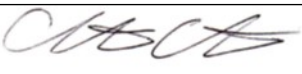


Guideline Title: Gastric Tube Insertion

Guideline Number: GUID-3203-PR010

Issue date: 09/09/2014	Replaces Dept. Policy: Gastric Decompression PR010
Revision dates: 4/30/24	Developed by: Air Care Team
Approved by: Dr. Christopher Hunter, MD Air Care Team Medical Director	Approved by:
Signature: 	Signature:
Department Numbers: 3203	

- I. **PURPOSE:** To effectively remove the gaseous and liquid contents of a patient's stomach via a nasally (NG) or orally (OG) placed tube. Also provides a means of administering enteral medications to a patient otherwise unable to ingest them.
- II. **DEFINITIONS:** None
- III. **DEPARTMENT GUIDELINE:**
 - A. **EQUIPMENT:**
 1. Personal protective equipment (PPE)
 2. NG/OG tube, appropriate size
 3. Water soluble lubricant
 4. 60 ml catheter tip syringe
 5. Tape or other securing device
 6. Stethoscope
 - B. **INDICATIONS:**
 1. Gastric emptying or gastric decompression
 - a. comatose, gastrointestinal bleeding, overdose, gastric distention/obstruction, cardiac arrest, intubated patient/advanced alternative airway with a port
 - C. **CONTRAINDICATIONS:**
 1. Absolute contraindications
 - a. Esophageal rupture, caustic ingestion (alkaline), nasogastric placement with nasal or midface trauma (orogastric tube may be placed), recent esophageal or throat surgery, recent nasal surgery (orogastric tube may be placed).
 2. Relative contraindications/considerations
 - a. Recent banding for esophageal varices, esophageal varices, esophageal stricture, coagulation abnormality (if known).
 - D. Be aware of complications: nasal bleeding, gagging, airway obstruction, placement in trachea
 - E. **PROCEDURE: Orogastric (OG)**
 1. Preferred route in intubated patients.

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2. Measure the OG tube alongside the patient. The end of the tube should be placed at the corner of the mouth, to the tip of the ear, and extended to the xyphoid process. This is the depth the tube should be inserted.
3. Insert tube into the oral cavity and advance into the stomach.
4. After the tube has been inserted, assess the tube for correct placement. A 60 ml syringe should be attached to the tube and gastric contents should be aspirated. 20 ml of air should be instilled into the abdomen while listening or feeling for an “air rush” over the patient’s abdomen.
5. Once gastric placement is confirmed, secure the tube in place with tape.
6. The tube should not be clamped if patient is to be transported by aircraft.

F. PROCEDURE: Nasogastric (NG)

1. Explain the procedure to the patient.
2. Measure the NG tube alongside the patient. The end of the tube should be placed at the tip of the nose, extended to the corner of the ear, and then to the xyphoid process. This is the depth the tube should be inserted.
3. Lubricate the end of the tube and begin to insert it into one of the patient’s nares. Pass the tube gently along the floor of the nasal passage and advance it as the patient swallows. Remove the tube if patient coughs or experiences dyspnea.
4. After the NG tube has been inserted, assess the tube for correct placement. A 60 ml syringe should be attached to the tube and gastric contents should be aspirated. 20ml of air should be instilled into the abdomen while listening or feeling over the abdomen for an “air rush”.
5. Once gastric placement is confirmed, secure the NG tube in place with tape.
6. The NG tube should not be clamped if patient is to be transported by aircraft.

G. Post placement

1. Consider hooking OG/NG to low intermittent suction or review orders for suction.
2. Measure and record amount of output, color, and consistency.

IV. DOCUMENTATION:

- A. EMS Charts

V. REFERENCES:

- A. None